# Operating Department Standard Of Practice YG

# ODSOP No: 25. Management of the Death of a Patient in Theatres YG

*Aligned to LOCSSIP –Standard 1;2;3;5;13*

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Aim- To continue care and maintain the dignity of the deceased patient.
To support religious and cultural beliefs held by the patient and their relatives.
To comply with legal and health and safety requirements.

**Any religious needs or cultural customs required for the care of the deceased or their relatives, must be respected**
**Guidelines for meeting the religious & spiritual needs of patients are available in theatre 4's yellow ODSOP file, the 611 file and the team leaders ODSOP file**
**The hospital chaplain (contacted via switchboard) will have links to appropriate religious leaders.**

## Scope
All Theatre Staff

## Responsibilities
- Theatre manager

Will ensure that the department and its staff are prepared for the appropriate management of deceased patients and their relatives to fulfil religious and cultural beliefs and to meet statutory health and safety legislation and legal requirements

- Team Leaders

Must ensure that their team members are aware of BCUHB and Departmental standards of care required for deceased patients and their relatives

- Theatre Staff

Will ensure an appropriate standard of care and management is given to the deceased patient and their relatives.

## Last Offices/Bereavement Box
- A yellow Last Offices/Bereavement Box containing equipment and relevant documentation can be found in the main theatre pack room in Y.G.
- When used, this must be replenished as required.
- In Llandudno General Hospital, contact Hospital Porters for last offices box to be brought to the department.

## Documentation
- Death must be confirmed/diagnosed as per AoMRC guidance and documented in the patients' medical notes. The Dr who confirmed the death must ensure their name and bleep number is **clearly** printed in the medical notes.

- An entry is also to be made in the theatre register/computer and theatre care plan notes.
- The proceedings prior to death are also to be recorded in the patient's medical notes and theatre care plan
- The Notification of Death form to be completed by nursing staff, and placed onto the front of the patients' notes.
- When family/significant others have been informed of the patients' death, this must be documented in the patients' notes where the date and time of death has been entered. and also clearly documented on the notification slip attached to the patients notes.

**Communication**

The person in charge of the theatre must immediately inform:-

- The Theatre Department manager or designated deputy
- The patients ward / ITU or Emergency Department staff.
- The 019 bleep holder (for Main Theatres YG)

Out of hours-

- The bleep 100 holder (Y.G.)
- The bleep 706 holder after 5pm (Llandudno G.H.)

- General porters are to be informed when the deceased patient is to be taken to the mortuary.

Medical staff must inform:-

- Their senior colleagues.
- The family of the patient's death at the earliest opportunity. (This task may be undertaken by senior nursing staff).
- Her Majesty's Coroner (as all deaths in theatre require referral to the Coroner).
- It is the responsibility of the doctor to inform the family of this procedure and of the possibility of a post mortem examination to establish the cause of death.

**Procedural Response to Unexpected Death in Children- PRUDIC**

- If a young person dies unexpectedly whilst an inpatient on an adult based ward or environment then the PRUDIC process will need to be followed diligently.
- This procedure is available on the Safeguarding children web page on the Intranet. The on call Paediatrician must be informed as per PRUDIC procedure and they will take a lead if this situation arises.
- The Safeguarding Liaison Nurse in the DGH will need to be informed promptly of this occurrence, as they will provide support to the staff and assist them in the management of the process

**Equipment management**

- If there is suspicion of an equipment failure or hazard affecting theatre, a decision may be made to take the theatre or anaesthetic machine out of commission until further notice.
- Also all anaesthetic equipment used for that patient including drug syringes and ampoules should be kept and stored in a secure place, should further investigation as to cause of death be required (AAGBI 2005 See reference below)

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## Care of the Deceased Patient

- **If there is any possibility of a police investigation regarding the death of the patient e.g. road traffic accident or industrial accident, or if the death is suspicious and/or unexplained, do not remove any endotracheal tubes, I.V. lines etc.**

- **If there is any prospect of a police investigation into the death, advice must be sought from a senior member of theatre staff, and the decision documented, regarding washing the patient, as washing may remove important forensic evidence. Staff MUST be certain prior to any washing or further procedures are undertaken to the body, that there will be no police involvement in the case**

- Infection control standards to be maintained during care of the deceased patient as per departmental ODSOP No. 28 Theatres Infection Control. Standard Precautions doc.
- When the patient is known or suspected to be an infection risk, in hours contact the infection prevention team' or when out of hours contact the on call medical microbiologist for advice on safe transportation of the deceased patient from theatres.
- For bodies infected with pathogens that may pose a serious risk to health, a completed risk category form must be attached to the patients medical notes and a label identifying the type of infection risk in the clear pocket of the cadaver bag.
- If the above situation occurs or when there is likely to be leakage of body fluids during transportation of the body, a body bag should be used
- Request for a bag can be made out of hours via the bleep 100 holder.
- Tracheostomy and endo-tracheal tubes are to remain in situ but *may* be removed **if** the doctor who confirms death is able to issue a Medical Certificate of Cause of Death. i.e. the death is due to natural causes.
- If removed, the position of the tube **must** be verified by a second anaesthetist not involved in the case, the removal **documented**, and the tube sent with the body to the mortuary.
- If a patient has been intubated for a period of time. e.g. on ITU or there are no prospects of legal proceedings, the **documented** opinion of one anaesthetist would be sufficient.
- All other drainage tubes including naso-gastric tubes, lines, surgical drains, chest drains or catheters etc should be left undisturbed **or managed as above.**
- If Catheters or cannulae are to remain in situ they must be closed with a spigot.
- Stoma bags should be renewed
- Apply a clean waterproof dressing to any wound(s).
- Referring to the patient's case notes the patient should be correctly identified by placing an identity bracelet on one wrist and one ankle on different sides of the body..
- If the deceased has a missing limb the identity bracelets should be placed on the same side of the body.
- Bar coded electronic identity bracelets may be used.
- When non electronic identity bracelets are used, details must be **clearly** written in capital letters in black pen. Details to include- full name: address: hospital D number: date of birth.
- Last offices to be performed as per Marsden Hospital Guidelines
- Please refer to the checklist in the yellow bereavement box.
- Tape a completed mortuary card to the shroud at chest level.

- Wrap the deceased patient in a sheet.
- Tape a mortuary label to the outer sheet bearing the patients full name, address and D Number or when a body bag is used attach this label to the outside of the bag.
- Also record on label if the patient has the following:-
  Cardiac Pacemakers. Joint replacements. Implantable Cardioverter Defibrillators or has been injected with a radioactive isotope.
- Once last offices are completed the nurse in charge will arrange as soon as possible transfer to the mortuary by calling for a general porter (phone 4030).*The verbal code BTM (body to mortuary) should be used.*
- The nurse **must** ensure the correct identity of the body before allowing transfer to the mortuary).
- General portering staff will bring the mortuary trolley to theatre recovery/reception, via the main corridor. Consideration must be given as to the activity in main reception/recovery prior to calling for the porter as patients may need to be delayed until transfer of the deceased patient from the theatre dept is completed.
- The patients' case notes, with death notification slip attached, and any relevant documents are sent with the body to the mortuary, in a sealed envelope.

## Management of the patients personal belongings

- All to be identified, checked and listed in the Patients Property book and the entry signed for by 2 trained members of staff.
- Jewellery should not be left on the patient unless otherwise instructed by a relative
- If jewellery such as a wedding ring or body piercing is left in situ, it is to be secured with tape, **documented** and signed for, in the nursing notes and on the mortuary card and Patient Property Book
- Any other valuables such a watches, bank cards, cash, mobile phones, keys,etc are to be sent to general office with completed copies of the Property Book sheet.
- Out of hours- Contact general porters and arrange to meet at general office where the hospital night safe is located, to deposit valuables and copies of documentation
- Clothing is sent with the patient to the mortuary, in a labelled bag.
- Clothing may be passed to next of kin when its receipt is acknowledged and signed for, in the Property Book.

## Care of the 'Organ Donor' Patient

- An organ donation reference box is available in theatre 4.
- For donor referrals and advice phone the **24 hr pager** 03000 20 30 40 and ask that the on call Specialist Nurse for Organ Donation rings theatre – always leave your name, the hospital name and the *full* contact number of the hospital including the extension you wish to be contacted on.
- The In house Specialist Nurse for Organ Donation is Abi Roberts 07889304437
- The In house Clinical Lead for Organ Donation is Pierre Payrass
- The identified link nurse within the dept is Caroline Evans
- The general principles of care as stated in the above ODSOP, will apply.
- The Specialist Nurse for Organ Donation will work with theatre staff to ensure all local policies/procedures and guidelines for care of the deceased patient are followed.

## Information and support to relatives

- If required the BCUHB Chaplain and/or Bereavement Officer can be contacted on Ext 5982/4169 or via switchboard.
- Relatives may request time to be with the deceased and their needs should be respected
- The theatre environment however is not always conducive to this need, and relatives should be advised that it may be more suitable for them to spend time with the patient in the bereavement unit.
- If they ask to see the deceased they **must** be made aware beforehand of tubes etc that may still be in situ
- In the likelihood that medical devices etc may be left on the body, relatives should be encouraged to view after the post mortem if possible, particularly if there is forensic interest in the case
- A Bereavement Booklet is available for relatives
- These are kept in the departments' "Bereavement Box".
- To obtain new booklets in YG contact general office Ext 4208 in LLGH contact Ext 2310
- Relatives who stay in the department must have their names entered in the visitors' book.

## Support of staff

- Following the death of a patient, a team debrief must be held during which a decision is to be made following principles of good practice, as to whether to cancel patients remaining on that list
- The decision to cancel patients must be clearly and promptly communicated to all relevant persons, wards and departments as per ODSOP No:18. Operating List Management
- Names of members of staff involved are to be noted by the team leader / 611 or 019 bleep holder and given to the theatre manager to enable further theatre team' de-briefing' sessions to be held as necessary
- The Chaplain and Bereavement Officer is available as above
- Any concerns regarding death or the systems of support in place for staff, must be reported to the bereavement nurse as soon as possible.
- Bereavement support is available from the Bereavement Officer as above
- Emotional and Counselling Support is also available through the Occupational Health and Wellbeing Service. Please contact **01745 448277** or **1815 8277** during office hours or leave a message on the 24-hour voicemail service on **02920 502 168.**

## Adverse Incident Reporting System

- Any death that occurs in theatre is considered a MUST REPORT
- The BCUHB electronic DATIX adverse incident reporting system is to be followed

## References


- BCUHB Policy IPC32 Last Offices: Care of the Deceased Patient (Infection Prevention management of deceased) Jan 2018
- BCUHB Policy IPC03 Standard Precautions Procedure Dec 2015
- BCUHB Policy IPC06 Hand Hygiene August 2016

- BCUHB Policy F01 Patient Property Procedure July 2012
- AfPP Standards & Recommendations for Safe Peri-operative Practice 2016
- Royal Marsden Hospital Last Offices Guidelines.
- AAGBI. Catastrophes in Anaesthetic Practice. Dealing with the Aftermath Sept 2005